



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : صباح عبدالنبي سيد متولى
المحافظة : القاهرة
الحى / المدينة : عين شمس
رقم البطاقة : 27203121401703 رقم التليفون : 01030073211
العنوان : عين شمس
العمر (تاريخ الميلاد) : 12-03-1972 المهنة : بدون عمل
تاريخ تسجيل الإستمارة : 30-09-2026

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (.....)
المقر بما فيه
التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مسح ذري علي الجسم [8530] قسم العلاج : الاورام
التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة:
الوظيفة:
الوظيفة:
التوقيع:
التوقيع:
التوقيع:
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / د. / د. / د. /

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
جهة العلاج :
ملاحظات :
قسم :
نفقات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: صباح عبدالنبي سيد مهدي
National ID: 27203121401703



تقرير طبي

المسجد

تحية طيبة وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الدخول :
-- -- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد انه

المریضة تعاني من ورم غير حميد بالبدن و تحتاج الي Whole-body PET/CT scan

الوقت :

-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين

جمهورية مصر العربية
بطالة تحليل الشخصية



صباح
عبد النبي سيد منكلى
عرب العرايدة
مركز الخفكة - القليوبية

١٩٧٢/٠٣/١٢ ٢٧٢.٣١٢١٤.١٧.٣
KT4062768

DMS
تاكسون

٢٠١٤/٠٩ ٢٧٢.٣١٢١٤.١٧.٣
بنون عل

أنتى
معلمة
مسترجة
معلمى حسن ملامه محمد
البطاقة سارية حتى ٢٠٣١/٠٩/٠٧




١١

Patient Name : الأستاذ / صباح عبد النبي سيد
Patient No. : 27203121401703
Path No. : 3498/25
Sex : Female
Age : 53 Y
Referring Doc :

Request No. : 13003275
Request Date : 12/11/2025
Verification Date : 18/11/2025
Financial Category : جوسف روسي - نفقة الدولة
Department : Out Patient
Registered by : marwa mohamed youss

Pathology Report

Clinical Data

Left breast mass.

Nature of specimen

Tru cut biopsy collected on 12/11/2025

Gross examination

Fragmented grey white tissue core ranging in size from 0.3 cm up to 1 cm, totally submitted.
Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:

HER2/NEU (4B5), Ki67 (30-9), PR (1E2), ER (SP1).

Microscopic examination

- * Histological type: Invasive duct carcinoma , focally necrotized .
- * Grade (differentiation +nuclear pleomorphism+ mitosis): (3+3+2) Grade 3
- * DCIS: Positive , solid , cribriform subtypes , intermediate & high grade
- * Lobular intra epithelial neoplasia : Absent
- * Angioinvasion: Positive
- * Microcalcification : Absent.
- * Perineural invasion: Absent
- * IPT (Positive control showed appropriate reactivity) :
- * ER: No nuclear reaction in tumor cells (Allred score 0/8)
- * PR: No nuclear reaction in tumor cells (Allred score 0/8)
- * HER2: Faint incomplete membrane reaction in <10 % of tumor cells (ultra-low)
- * Ki 67: 80 %

Verified by :

Dr. Heba Hasan

Printing Date : 24/11/2025 11:05:58

Page 1 of 3

Printing By : marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor - Building No. 987 - Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name: SABAH ABD ELNABY SAYED 52Y	Patient ID: 27203121401703
Report Date: 16/09/2026 12:02:45 PM	Age: 52 Years, 8 Months, 15 Days

DYNAMIC MRI STUDY OF THE ABDOMEN

Technique of examination:

- Axial T1 (in-out phases) & T2 WIs .
- Coronal T2 WIs .
- Post contrast axial T1 WIs.
- Axial DWI & ADC

Findings:

- Enlarged sized liver measuring 17.6 cm with no pathologically enhancing focal lesions.
- No dilatation of the intra hepatic-biliary radicals.
- Muddy gall bladder with average distention and no stones.
- Patent homogenously enhancing portal vein and its main branches.
- Normal size & MR signal of the spleen, with no focal lesions within its parenchyma.
- Multiple bilateral renal High T2 simple cortical cysts largest at right kidney upper pole measuring 3.15 X 3.11 X 2.52 cm, otherwise Normal site, size and parenchymal thickness of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, IVC and both supra-renals.
- No ascites.
- No pathologically enlarged abdominal lymph nodes.
- Normal MRI appearance of the lung bases.

Conclusion:

- ❖ *Mild hepatomegaly with no focal lesions as described.*
- ❖ *Muddy Gall bladder .*
- ❖ *Bilateral renal simple cortical cysts.*

Dr.Ahmed Ashraf

Dr. Ali Elsaied, MD, EFR, EBIR

Dr. Ayman Elsayed



Patient's Name: Sabah A ayed

Date: 2026-09-14

Referred by: Prof.Dr. no doctor

Hospital: Goustave Roussey International Egypt

Bilateral full field digital Mammography with high-resolution breast sonography revealed:

Clinical summary: Pathologically proven left breast cancer on NACT for assessment.

Findings:

Right breast.

- dense nodular breast parenchyma (ACR c)
- scattered benign calcification
- No speculations, masses, asymmetry, distortion or suspicious microcalcifications.
- U/S No solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Left breast.

- Subtle focal asymmetry seen at center and UOQ measuring about 40x28mm
- scattered benign calcification
- U/S between 11-2 o'clock shows il defined heterogeneous solid lesion measuring 44x26 mm (previously measured 33x20mm)
- normal skin thickness
- Enlarged one lymph node at level I measures 15x7mm with loss of fatty hilum with multiple indeterminant lymph nodes .

Conclusion:-

Right breast (BIRADS 2): adenosis

Left breast (BIRADS 6): partially radiological response regarding the previously detailed malignant process

Best Regards,

DR. Marwa M.Hassan

جولف
روستاق

ص.م. ٨١٦



مادة رئيس الجمهورية
لصحة المرأة المصرية

الجمهورية
البحرينية

08/06/2026

0

MDT FORM

Name:	Sabab Abd Elnaby Sayed Metwally	MRN:	3728
Age:	54 Y	ID:	27203124401703
Menopausal Sta		Phone:	0118073197
NRS SCORE:		Marital Status:	
BMI:		Family History:	
CO Morbidity:	DM, HTN CARDIAC, VALVE REPLACEMENT 10 YEARS	Reason of MDT:	

Clinical Data:

BX:	TNBC						
ER:	0/8	PR:	0/8	HER2:	-ve	KI67:	

R

Sonomammography:

53 y/o female, TNBC (ER 0/8, PR 0/8, HER2-), Ki-67 80%, initially presented with pulm mets (LLL nodules, largest 1.5 cm) + bone mets (D12, L1 WEDGED VERTEBRE IIII).

RECEIVED : 12 cycles wky paclitaxel 7 complicated by G3 peripheral neuropathy.

FU CT CAP (Apr 2016): marked regression with near CR of pulm nodules (now 25 mm), no new lesions.

T N M

Stable sclerotic lesions at D12 & L1. , BONE SCAN >> P

Mets Workup:

History:

postm left breast cancer met to lung ldc triple -ve ki 80 ct2 no m1 Initial cts 12/2025 pulm nodules largest 1.5 rec taxol cts 4/2026 rd of pulm nodules largest 5 mm met on 26/4/2026 in view of triple -ve and multiple pulm nodules which emphasise its met nature and responding to chemo to continue on taxol and monitor neuropathy .hba1c 9.3 for endocrine cons received weekly taxol 110 week21

patient complains of grade 3 PN on cymbatex 50mg ct cap 7 /2026 : stationary course + newly developed right HFLs 5mm

Recommendations:

MDT Outcome & Plan

Oligometes now CR (MAINTAIN CR SINCE 1/2026)
FOR
DYNAMIC MRI LIVER
PET-CT (JUSTIFY IT IN MOH AS PT OLIGOMETES NO CR NEED TO BE CONFIRMED)
COUNSELLING PT FOR SURGERY TO LT BREAST

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric



جمهورية مصر العربية
وزارة الداخلية
إدارة الهجرة

احمد
محمد اسماعيل بشندي
٢ ح الدكي - ش المحطة
مصر القديمة - القاهرة

١٩٦٧/١

٢ ٦٧ ١٠ ٢٢ ٢١ ٠ ٢٥ ٣٢

IR8321935



٢٠٢٠/١٢ ٢٦٧١٠٢٢٢١٠٢٥٣٢

سائق

ذكر

مسلم

متزوج

البطاقة سارية حتى ٢٠٢٧/١٢/٢١





جهة الاصدار : فرع القاهرة

تاريخ الاصدار : 2024-1-29

الى من يريه الامر

الهيئة العامة للتأمين الصحي تعمل من خلال قوانين منظمة للانتفاع الخاضعين لقانون التأمين الاجتماعي رقم 79 لسنة 1975 المعدل بالقانون 5 لسنة 1977 "القوى العاملة والمعاشات والارامل" وكذلك قانون 99 لسنة 1992 الخاص بطلاب المدارس و قانون رقم 86 لسنة 2012 "الإطفال دون السن المدرسي" وقانون رقم 23 لسنة 2012 الخاص بالمرأة المعيلة .

وطبقا لقاعدة البيانات المرسلة من التأمينات الاجتماعية
وطبقا لقاعدة البيانات المرسلة من الضمان الاجتماعي
والتحقيق فتبين أن

السيد (ة) احمد محمد اسماعيل يشندى بطاقم رقم 26710222102532
وطبقا الى خطاب من التأمينات الاجتماعية يفيد عدم الانتفاع
يعتبر من الفئات غير المستفيدة بالتأمين الصحي
للعامة للتأمين الصحي حال خضوع المواطن لقانون التأمين الصحي الشامل
شكرا

الموظف المختص
اسلام عبد الحميد

تاريخ الطباعة : 2024-1-29



إفادة مركز نظم المعلومات
بالتأمين



مسجل 1238801

الخطيب سحر محمد شادية أشهد من تاريخ الاصدار الا في حاله غير حاله المواطن من قبل الهيئة أفقوية للتأمينات الاجتماعية او
وإذا كان الشخص التأميني او ووزارة التربية والتعليم

الإدارة المركزية لتكنولوجيا المعلومات ودعم القرار

Patient Name: احمد محمد اسماعيل بشندي
MRN: 123019868
D.O.B: 22/10/1967
IC No.: 267000000000000

AGE: 57 Y 07 M 23 D
Location: امراض الدم
Sex: Male

HEMATOLOGY
BONE MARROW ASPIRATION

Specimen Id 2503100669
Accession No Hem-25-72747
Sample Type Bone marrow

Collection Date 10/03/2025 12:43 PM
Received Date 10/03/2025 01:57 PM
Doctor امراض الدم

PB Finding :

TLC Normal
RBCs Anemia
with rouleaux
Platelets Normal

BONE MARROW ASPIRATION

Site Right Posterior superior Iliac Spine

Overall marrow cellularity

Erythropoiesis Hypercellular for the pattern age
Granulopoiesis Erythroid hyperplasia
Megakaryopoiesis Normal granulopoiesis

Normoblasts : Normal megakaryopoiesis

Blasts : 45

Promyelocytes : 0

Myelocytes : 2

Juvenile : 8

Staff: 13

Segmented : 10

Eosinophils : 13

Basophils : 2

Lymphocytes : 0

Plasma Cells : 5

Monocytes : 2

Megakaryocytes with normal thrombopoiesis

Conclusion: A known case of Multiple myeloma follow up, BMA is mild hypercellular for age with 2%plasma cells for BMB for better assessment.

Signature:

Tested by	Approved by
Noha Muhammad Jaballah	Lucie Gamal Fawzy

Dr.
Elia Anis Ishak

M. Sc. Ph. D. Pathology - Prof. of Pathology , Cairo University
Fellow, Armed Forces Institute of Pathology , USA
Member, United States Canadian Academy of Pathology

Dr.
Shady Elia Anis

M. Sc. MD Pathology , Ass. Prof. of Pathology , Cairo University
Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : احمد محمد اسماعيل بشندي

Age : 56 Y : 1 M : 9 D

Referred By Prof. Dr. : طارق جبر

Clinical Diagnosis : D8 lytic lesion lesion.

Nature of Specimen : Resection

Receiving Date : 2024-08-31

Analysis ID : 882113

Gender : Male

Patient ID : 595130

PATHOLOGY REPORT

Gross :

Multiple bony tissue pieces, collectively measured 4x4 cm, totally submitted.

Microscopic :

Sections examined from the specimen received revealed bony tissue infiltrated by neutrophils, plasma cells and histiocytes within a collagenous back ground. Foci showed fibrosis with osteoclastic reaction (Evidence of old haemorrhage).
No malignancy in material received.

Diagnosis :

D8 lesion, Resection, MIXED ACUTE AND CHRONIC NON SPECIFIC INFLAMMATORY INFILTRATE INVOLVING BONE (OSTEOMYELITIS), NO RECURRENT TUMOR IN MATERIAL RECEIVED.

Prof. Dr. Shady Elia Anis

Prof. Dr. Elia Anis Ishak

Shady Elia

Elia anis

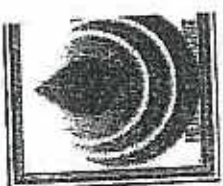


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**National
Cancer
Institute**
Cairo University

NCI, Cairo University
Clinical Pathology department
Hematology Unit



Patient Name	: احمد محمد اسحاق رشدي	Age	: 53 Y 1 M 27 D	Patient Number	:
Request Date	: 06-01-2024 13:38	Sex	: Male	Accession No.	:
Printed Date	: 27-01-2024 09:46	Department:	: دكتور	Barcode	:
Report Date	: 21-01-2024 12:08	Doctor	:	Doctor Telephone	:
Sample Type	: Bone Marrow	Diagnosis	:		
Address- Tel	: -				

Bone Marrow Biopsy (BM/B)

Test

Result

BMA Findings

PB(anaemia with mild rouleaux; high normal plt's with many aggregates, low TLC), BMA(high normocellular for age ,4% plasma cells).

BMB Gross examination

BM trephine biopsy core(s)

BMB Microscopic examination

Age related overall marrow cellularity & fat spaces

Hematopoiesis

Abnormal infiltrations

Reticulin fibers and collagen bundles contents & distribution

BM trabeculae

Immunohistochemistry

Conclusion

Unilateral, Adequate multiple cores

Normocellular for the patient's age

.represented
mild interstitial increase in plasma cells.
1-2

Normal Trabeculae

CD138(5-10%).

Known case of MM, normocellular for age showing mild interstitial increase in plasma cells.

Signature

Tested By	Approved by	Scientific Supervisor
Dr. Lamyaa Fadaly	Dr. / Nevlin Alazhary	

F-NCI-CPD-HEM-QAU-TP014/01

Date of Issue

Patient Name: Ahmed Muhammed Ismail B		AGE: 58 Y 07 M 02 D
MRN: 123019868	Location: امراض الدم	عيادة
D.O.B: 22/10/1967	Sex: Male	
IC No.: 2670000000000000		

HEMATOLOGY BONE MARROW TREPINE BIOPSY

Specimen Id	2605030932	Collection Date	03/05/2026 12:16 PM
Accession No	Hem-26-178660	Received Date	03/05/2026 12:32 PM
Sample Type	Bone marrow	Doctor	امراض الدم

Specimen number: 462

PB Findings : Normal TLC, anemia, normal platelets.

BMA Findings : Normocellular for age, 1% blasts.

Site: PSIS

BMB Gross examination:BM trephine biopsy core(s): Unilateral, multiple cores (3, 1 cm), adequate (>7 intertrabecular spaces).

Histological examination of BM trephine biopsy sections revealed:

Age-related overall marrow cellularity & fat spaces: High normocellular BM for age (Cell/fat ratio: 50%).

Hematopoiesis:

Erythroid: Normal distribution, count, morphology and maturation.

Granulocytic

Megakaryocytic

Abnormal infiltrations: Normal distribution, count, morphology and maturation.

Reticulin fibers and Collagen bundles contents & distribution: Normal distribution, count, morphology and maturation.

BM trabeculae: Mild interstitial increase in plasma cells.

Immuohistochemistry

MF-0: Scattered linear reticulin with no intersections (crossovers) corresponding to normal bone marrow

Normal

Was performed on formalin-fixed, paraffin-embedded tissue sections using the Dako EnVision™ FLEX detection system

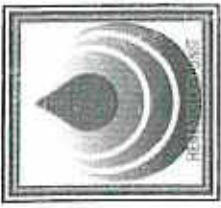
CD138: 10%

A case of MIM showing high normocellular BM for age with 10% plasma cells which highlighted b CD138.

Conclusion: Serum protein electrophoresis, immunofixation, quantitative Igs.

Further recommended investigation:

Signature:	Tested by	Approved by
	• Muhammad Mustafa Ahmed Fouad	نظير محمد فتحي حسين



Patient Name	: احمد محمد اسماعيل رشدي	Age	: 56 Y 4 M 22 D	Patient Number	: 12301986
Request Date	: 31-03-2024 09:49	Sex	: Male	Accession No.	: 50690
Printed Date	: 22-04-2024 08:55	Department	: بزل عجز	Barcode	: 34000419
Report Date	: 18-04-2024 11:15	Doctor	:	Doctor Telephone	:
Sample Type	: Bone Marrow	Diagnosis	:		
Address- Tel	: -				

Bone Marrow Biopsy (BMB)

Test	Result
------	--------

BMA Findings

PB(anaemia with rouleux,normal TLC,normal plts),BMA(high normocellular ,abundant megs,2% plasma cells)

BMB Gross examination

BM trephine biopsy core(s)

Unilateral, Adequate multiple cores

BMB Microscopic examination

Age related overall marrow cellularity & fat spaces

Hypercellular for the patient's age

Hematopoiesis

.represented ,increase megs

Abnormal infiltrations

mild interstitial increase in plasma cells

Reticulin fibers and collagen bundles contents & distribution

.0-1

BM trabeculae

Normal Trabeculae

Immunohistochemistry

CD138(10-20%)

Conclusion

Known case of MM ,hypercellular for age showing mild interstitial increase in plasma cells.

Signature

Tested By	Approved b	Scientific Superviso
Dr.Lamyaa Fadaly	Dr \ Nevin Alazhary	

F-NCI-CPD-HEM-QAU-TP014/01

Date of Issue

Dr.
Elia Anis Ishak

M. Sc. Ph. D. Pathology - Prof. of Pathology , Cairo University
Fellow, Armed Forces Institute of Pathology , USA
Member, United States Canadian Academy of Pathology

Dr.
Shady Elia Anis

M. Sc. MD Pathology , Ass. Prof. of Pathology , Cairo University
Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : احمد محمد اسماعيل بشنوي

Age : 56 Y : 1 M : 9 D

Referred By Prof. Dr. : طارق جبر

Clinical Diagnosis : D8 lytic lesion lesion.

Nature of Specimen : Resection

Receiving Date : 2024-08-3

Analysis ID : 882113

Gender : Male

Patient ID : 595130

PATHOLOGY REPORT

Gross :

Multiple bony tissue pieces, collectively measured 4x4 cm, totally submitted.

Microscopic :

Sections examined from the specimen received revealed bony tissue infiltrated by neutrophils, plasma cells and histiocytes within a collagenous back ground. Foci showed fibrosis with osteoclastic reaction (Evidence of old haemorrhage).
No malignancy in material received.

Diagnosis :

D8 lesion, Resection, MIXED ACUTE AND CHRONIC NON SPECIFIC INFLAMMATORY INFILTRATE INVOLVING BONE (OSTEOMYELITIS), NO RECURRENT TUMOR IN MATERIAL RECEIVED.

Prof. Dr. Shady Elia Anis

Prof. Dr. Elia Anis Ishak

Shady Elia

Elia Anis



This report is electronically signed , any abrasion or modification would render it void .

Patient Name: احمد محمد اسماعيل بشندي
 MRN: 123019868
 D.O.B: 22/10/1967
 IC No.: 2670000000000000

AGE: 58 Y 07 M 02 D
 Location: الدم
 Sex: امراض Male
 عيادة

HEMATOLOGY BONE MARROW ASPIRATION

Specimen Id 2605030938
 Accession No Hem-26-178664
 Sample Type Bone marrow

Collection Date 03/05/2026 12:17 PM
 Received Date 03/05/2026 12:36 PM
 Doctor امراض الدم

PB Finding :

TLC Normal
 RBCs Anemia
 Platelets Normal

BONE MARROW ASPIRATION

Site Right Posterior superior Iliac Spine

Overall marrow cellularity

Erythropoiesis Normocellular for the patient age
 Granulopoiesis Erytheroid hyperplasia
 Megakaryopoiesis Normal granulopoiesis
 Normoblasts : Normal megakaryopoiesis

Normoblasts : 33

Blasts : 0

Promyelocytes : 13

Myelocytes : 17

Juvenile : 8

Staff: 14

Segmented : 4

Eosinophils : 2

Basophils : 0

Lymphocytes : 8

Monocytes : 0

Plasma Cells : 1

Conclusion:

A referred case of MM, showing low normocellular bone marrow for age with no abnormal plasma cells.
 for correlation with BMB.

Signature:

Tested by

Hend Abdel Hadi Noah Mahmoud

Approved by

Hend Abdel Hadi Noah Mahmoud



ent Name:	احمد محمد اسعيل بشاري	AGE:	58 Y 04 M 08 D
V:	123019868	Location:	الدم
B:	22/10/1967	Sex:	Male
Id:	2670000000000000		

HEMATOLOGY
BONE MARROW ASPIRATION

Specimen Id	2601190981	Collection Date	19/01/2026 10:45 AM
Session No	Hem-26-27921	Received Date	19/01/2026 01:41 PM
Sample Type	Bone marrow	Doctor	امراض الدم

Finding:

Normal
Anemia
with mild rouleaux
Normal
platelets aggregates were seen

IE MARROW ASPIRATION

Normal marrow cellularity

Normocellular for the patient age

Normal erythropoiesis

Normal granulopoiesis

Normal megakaryopoiesis

Normoblasts: 20

Myeloblasts: 0

Myelocytes: 3

Eosinophils: 3

Neutrophils: 3

Lymphocytes: 7

Monocytes: 33

Plasma cells: 3

Atypical lymphocytes: 0

Plasmacytoid lymphocytes: 22

Atypical plasma cells: 0

Normal plasma cells: 6

Atypical monocytes: 0

Conclusion: Known case of MM showing normocellular bone marrow for age, 6% plasma cells for correlation with BMIB for better assessment of plasma cells count.

Signature:

Tested by	Approved by
Nada Nabawi Hamed Gharib	Luiza Gamal El-Dokkany

Patient Name: Ahmed Muhammed Ismail B		AGE: 57 Y 07 M 23 D
MRN: 123019868	Location: امراض الدم	عيادة
D.O.B: 22/10/1967	Sex: Male	
IC No.: 267000000000000		

Hematology
BONE MARROW TREPINE BIOPSY

Specimen Id 2503100668
Accession No Hem-25-72746
Sample Type Bone marrow

Collection Date 10/03/2025 12:51 PM
Received Date 10/03/2025 01:57 PM
Doctor امراض الدم

PB Findings :
Anemia with rouleaux formation, normal ltc and platelets with aggregates

BMA Findings :
Hypercellular marrow for age with erythroid hyperplasia, normal granulopoiesis, thrombopoiesis and 5% plasma cells with rare binucleated forms seen on scanning

Site:
Normal

BMB Gross examination:BM trephine biopsy core(s):
Unilateral, adequate multiple cores

Age-related overall marrow cellularity & fat spaces:
High normocellular marrow for age

Abnormal infiltrations:
Represented trilineage hematopoiesis with increased megakaryocytes

Reticulin fibers and Collagen bundles contents & distribution:
Interstitial increase in plasma cells

BM vascularity (Angiogenesis):
Grade 0-i

BM trabeculae:
Normal

Others:
A known case of multiple myeloma showing high normocellular marrow for age with interstitial increase in plasma cells (around 15-20%) as confirmed by IHC.

Immuohistochemistry
CD138: Positive around 15-20%

Conclusion:

Signature:

Tested by	Approved by
May Mostafa Abdel Fattah	Muhammad Abdel Mohsen Zaki Mustafa

Dr.
Elia Anis Ishak

M. Sc. Ph. D. Pathology - Prof. of Pathology , Cairo University
Fellow, Armed Forces Institute of Pathology , USA
Member, United States Canadian Academy of Pathology

Dr.
Shady Elia Anis

M. Sc. MD Pathology , Ass. Prof. of Pathology , Cairo University
Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : احمد محمد اسماعيل بشندي

Age : 56 Y : 1 M : 9 D

Referred By Prof. Dr. : طارق جبر

Clinical Diagnosis : D8 lytic lesion lesion.

Nature of Specimen : Resection

Receiving Date : 2024-08-31

Analysis ID : 882113

Gender : Male

Patient ID : 595130

PATHOLOGY REPORT

Gross :

Multiple bony tissue pieces, collectively measured 4x4 cm, totally submitted.

Microscopic :

Sections examined from the specimen received revealed bony tissue infiltrated by neutrophils, plasma cells and histiocytes within a collagenous back ground. Foci showed fibrosis with osteoclastic reaction (Evidence of old haemorrhage).
No malignancy in material received.

Diagnosis :

D8 lesion, Resection, MIXED ACUTE AND CHRONIC NON SPECIFIC INFLAMMATORY INFILTRATE INVOLVING BONE (OSTEOMYELITIS), NO RECURRENT TUMOR IN MATERIAL RECEIVED.

Prof. Dr. Shady Elia Anis

Prof. Dr. Elia Anis Ishak

Shady Elia

Elia anis



This report is electronically signed , any abrasion or modification would render it void .

Patient's Name: Sabah Abdelnabi El-Sayed
Date: 2026-07-16
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

Compared to the previous CT scan performed on 2-4-2026, the current study revealed:

I- CHEST:

- No appreciable time interval changes regarding the size and number of the previously noted bilateral pulmonary nodules the largest is seen in the left upper lobe apical segment measuring 5 mm
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Sternotomy sutures with marginal calcification of the left atrial wall and dilated right atrium ...for echo correlation

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No or dilated biliary radicles. Newly developed right hepatic lobe small enhancing lesion measuring 5 mm seen at segment VIII
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulac or stones.
- Still noted Wedged vertebral bodies of DV12 and LV1
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

Opinion:

- Newly developed small right hepatic lobe focal lesion.
- Stationary course regarding the rest of the study

Patient's Name: Sabah Abdelnabi El-Sayed
Date: 2026-07-16
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

Dr.Asser Ahmed

MULTISLICE C.T. EXAMINATION OF THE NECK **AFETR IV CONTRAST INJECTION REVEALED:**

- Normal appearance of the naso-, oro- and hypopharynx with preserved parapharyngeal fat planes.
- Intact laryngeal skeleton with no glottic, supra or infra glottic masses.
- Normal course and calibre of the carotid and jugular vessels.
- Normal CT appearance of the submandibular and parotid salivary glands showing homogenous enhancement. No focal solid or cystic lesions.
- Few subscentemetric bilateral upper deep cervical and submandibular LNs are noted; likely non-specific.
- No pathologically enlarged cervical lymph nodes.

Opinion:

- Normal C.T study of the neck.

Dr.Asser Ahmed

Dr.Asser Ahmed

Patient's Name: sabah a naby sayed metwally
Date: 2026-04-02
Referred by: Prof.Dr. ct cap iv
Hospital: Goustave Roussey International Egypt

**MULTISLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN
AND PELVIS REVEALED:**

Compared to the previous CT scan performed on 6-12-2026, the current study revealed:

I- CHEST:

- Marked regression mounting to total resolution regarding the size and number of the previously noted bilateral pulmonary nodules the largest is seen in the left upper lobe apical segment measuring 5 mm
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Sternotomy sutures with marginal calcification of the left atrial wall and dilated right atrium ...for echo correlation

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal size, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Still noted Wedged vertebral bodies of DV12 and LV1
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

Opinion:

- Regressive course regarding the previously noted bilateral pulmonary nodules
- Stationary course regarding the rest of the study

Dr. Asser Ahmed



مستحق
أ. ش. حيدر
١١/١١/٢٠٢٢



١١/١١/٢٠٢٢
١١/١١/٢٠٢٢
١١/١١/٢٠٢٢

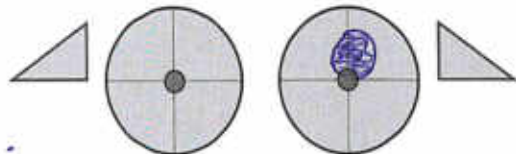


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١١/١١/٢٠٢٢

MDT FORM 25 / 12 / 20 25 G

<u>Name</u>	Amir Zafar	<u>MRN</u>	3488/2025
		<u>ID</u>	CCXICIS-14
<u>Age</u>	53 yo	<u>Phone</u>	0103007321
<u>Menopausal status</u>	post	<u>Marital status</u>	Married
<u>NRS SCORE</u>		<u>Family history</u>	-ve
<u>BMI</u>		<u>REASON OF MDT</u>	New Case
<u>CO morbidity</u>	HTN/ D.M/ valve replacement.		

Clinical Data

<u>BX:</u> IDC G III				R	L
<u>ER:</u> 0/8	<u>PR:</u> 0/8	<u>HER 2:</u> Neg	<u>Ki67:</u> 80%		
<u>Sonomammography:</u> Lt 90Q mass. Periareolar 1-2 o'clock 3.3x3x1.2cm. Lt axillary reactionary LN 2.5x9mm ??					
<u>Mets Workup:</u> CT CAP → few pulm. nodules at Lt lower lobe. 1.5cm. (<u>Mets</u>) wedged vertebral bodies of D12 & L1				T N M 2 0 1	

Recommendations:

for WACT

راہانی شہر

(*) Taxol weekly

MDT OUTCOME & PLAN

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. احمد الفراء د. فتيحة صبيح 14/10/2019					

Patient Name	: الأستاذ / صباح عبد النبي سيد	Request No.	: 13003275
Patient No.	: 27203121401703	Request Date	: 12/11/2025
Path No.	: 3498/25	Verification Date	: 18/11/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 53 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data
Left breast mass.

Nature of specimen
Tru tut biopsy collected on 12/11/2025

Gross examination

Fragmented grey white tissue core ranging in size from 0.3 cm up to 1 cm, totally submitted.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against: HER2/NEU (4B5), KI67 (30-9), PR (1E2), ER (SP1).

Microscopic examination

- * Histological type: **Invasive duct carcinoma**, focally necrotized.
- * Grade (differentiation + nuclear pleomorphism + mitosis): (3+3+2) Grade 3
- * DCIS: Positive, solid, cribriform subtypes, intermediate & high grade
- * Lobular intra epithelial neoplasia: Absent
- * Angioinvasion: Positive
- * Microcalcification: Absent
- * Perineural invasion: Absent
- * IPT (Positive control showed appropriate reactivity):
- * ER: No nuclear reaction in tumor cells (Allred score **0/8**)
- * PR: No nuclear reaction in tumor cells (Allred score **0/8**)
- * HER2: Faint incomplete membrane reaction in <10 % of tumor cells (**ultra-low**)
- * Ki 67: **80 %**

Verified by :
Dr. Heba Hasan

Printing Date : 24/11/2025 11:05:58

Page 1 of 3

Printing By : .marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor - Building No. 987 - Nile Corniche - Misr El Qadema - Cairo, Egypt

RADIOLOGY REPORT

Name: Sabah Abdulnaby

Age: 50 Y

Ref. Prof. Dr. / Moamen Elmashtouli

Date: 28/10/2025

Left Breast US

- Left breast shows a slightly defined lobulated irregular strongly hypo-echoic movable soft tissue mass lesion, located in peri-areolar region of upper outer quadrant at 1 & 2 O' clock = 33 x 30 x 19 mm in size with few internal calcifications & posterior acoustic enhancement.
- No duct ectasia or nipple retraction seen in both sides.
- Left axillary reactionary looking multiple enlarged lymph nodes seen, the largest = 25 x 9 mm.

CONCLUSION:

- Picture of left breast mass lesion...(BIRADS 4a)...for further mammography & biopsy.

Much obliged, Radiology consultant
Signature, *[Signature]*
DR /

02-44682525 - 01550869711

٥ من السبت للخميس ١ ظهراً إلى ١٢ مساءً و الجمعة من ٥ م إلى ١٠ م
٦ الخانكة - شارع ترعة السرجاني - برج الأطباء - الدور الثاني

Ministry of Health and Population
Specialized medical centers
EL-Khanka Specialized Hospital
Radiology Department, mammogram



وزارة الصحة والسكان
أسامة المراكز الطبية المتخصصة
مستشفى الخانكة التخصصي
قسم الأشعة / وحدة الماموجرام

Patient Name :- محمد عبد الله

Date :- 2021/1/12

age :- 60

Bilateral Breast Mammogram Revealed;

Right Breast And Axilla :- ACR / C

- * Cystic or mass lesions :- no
- * signs of calcification :- vascular calcifications
- * skin / nipple / areolar complex :- normal
- * axillary lymph nodes :- normal

Left Breast And Axilla :- ACR / C

- * Cystic or mass lesions :- upper quadrant ill defined irregular dense mass with tiny calcifications
- * signs of calcification :- vascular calcifications
- * skin / nipple / areolar complex :- normal
- * axillary lymph nodes :- normal

Conclusion :- * Lt. Breast mass lesion with Lt axillary LN
(BIRADS 4) → Biopsy

Radiologist:-

[Signature]
Dr. / [Signature]
رئيسة الأقسام
وحدة الأشعة التخصصية